Challenge Stigma | Find Peace

FrontlineFolks | Guide #27

Family Life in Transition: Moves, Shifts, and New Normals

A Field Guide for Frontline Folks and Their Families

Field notes for the pattern in this chapter.

By Michael Jenkins, MS, MSW, LCSW
Never Forgotten Services, PLLC

Disclaimer

This guide is for general education only.

It is not:

- Therapy
- Medical care
- A crisis service
- Individualized advice

Reading this guide does not create a therapist–client relationship with the author or with Never Forgotten Services, PLLC.

If you are in crisis or thinking about harming yourself or someone else, call 988 in the United States, go to your nearest emergency room, or contact your local emergency services.

If you want mental health treatment, please contact a licensed provider in your state or call the number on the back of your insurance card.

Table of Contents

1. What Is Happening
2. Why It Escalates
3. What To Do This Week
4. For Partners and Team
5. When To Get More Help
6. 7-Day Field Drill
7. Worksheets
    - Worksheet 1: Worksheet: Family Life in Transition: Moves, Shifts, and New Normals
    - Worksheet 2: Worksheet: Family Life in Transition: Moves, Shifts, and New Normals - Partner/Team

1. What Is Happening

Family transitions—moves, shift changes, returning from deployment, changing jobs, retirement—are times when your nervous system dysregulation becomes most visible and most harmful. Normally you manage it with routine. Transition wipes out routine. Everyone is stressed, nothing is familiar, and you have no downshift environment. Your dysregulation hits harder, and the family's secondary trauma amplifies. What was barely-managed suddenly becomes crisis.

Common transition stressors

- Geographic moves: New house, new neighborhood, kids in new school, partner's social network gone.
- Shift changes: Different sleep schedule, different family dinner time, different 'on' and 'off' hours.
- Returning from extended leave or deployment: Reintegration is harder than people think. Family has adapted to absence.
- Job changes or promotions: Different schedule, different stress, different physical location, different commute.
- Retirement or separation from frontline work: Loss of identity, loss of structure, loss of colleagues as anchors.
- New family members: New baby, step-parent, blended family—all creating new dynamics and stress loads.

> **Transition is normal, but dysregulation + transition is volatile:**
> Expect higher reactivity, lower patience, and more conflict during transitions. Plan for it instead of being blindsided.

2. Why It Escalates

During transitions, your nervous system loses familiar triggers and safe spaces. The routine that structured your dysregulation is gone. Your family loses their rhythm for managing you. Everyone is off-balance, expectations are unclear, and small stressors become explosions. This is when breakdowns happen, when kids act out, when partners become resentful.

Why transitions amplify dysregulation

- Loss of predictable structure: No familiar route home, no familiar pace, no familiar downshift environment.

- New stressors layered on old: The transition stress + your baseline dysregulation = volatile combination.
- Family system off-balance: Everyone's reacting to the transition, no one's managing your activation.
- Unclear expectations: Roles change mid-transition. Does partner still manage bedtime? Are kids responsible for packing? Who handles dinner?
- Isolation: In moves, your partner loses their support network. No one to vent to about managing your mood.
- New environments don't have the same safety cues: New house = new triggers, new routines = new confusion.

3. What To Do This Week

You can reduce the damage transitions cause by planning ahead, communicating expectations clearly, and intentionally building new stabilizing structures.

1. Before transition: Have one conversation mapping expectations, roles, and how you'll manage activation during the chaos.
2. Identify your new downshift ritual immediately: Before the move/shift change/etc., figure out what will be your regulating activity in the new normal.
3. Create one predictable anchor for kids: Same bedtime, same weekend ritual, same 'dad's not available time'—something kids can count on.
4. Check in with partner weekly during transition: 'How are you doing? What do you need from me? Am I showing up?'
5. Explicitly agree on what 'reduced parenting' looks like during transition: Some weeks you're in 'keep them alive' mode. That's okay.
6. Plan a hard reset one month after transition: New routine has been attempted, now debrief what's working and what needs to change.

> Field rule: set the bar low enough to do it even on your worst day.

4. For Partners and Team

Stress communication should be clear, brief, and repeatable. Predictable routines protect relationships.

- Use short, concrete language during transitions: 'I'm activated by the move. I'm not being clear. Here's what I need.'

- Agree on pause and return-time rules: Transitions are high-stress. Honor when partner or kids need space.
- Debrief after hard moments without blame: 'The transition is harder than I thought. Let's adjust expectations.'
- Coordinate communication: Partners stay on same page about new routines, rules, and expectations.

---

5. When To Get More Help

Self-guided tools are a start. Escalate care when symptoms persist, spread, or begin harming safety and function.

- Transition has been complete for three months, but dysregulation hasn't settled into new routine.
- Kids or partner are showing distress that's worsening instead of improving.
- Conflict has increased and repair efforts aren't working.
- You're noticing this is the third transition where things fell apart.
- Family-centered support (family therapy during transition, coached communication, trauma-informed planning) can prevent crisis and build resilient new normals.

6. 7-Day Field Drill

Use the worksheet every day for one week. Track trigger, body signal, and reset action. Keep what works and remove what does not.

> **Daily drill:**
> 1) Name the trigger.
> 2) Name the first body signal.
> 3) Run one reset action.
> 4) Log response quality.

Worksheet 1: Worksheet: Family Life in Transition: Moves, Shifts, and New Normals

Use this to track the pattern in this chapter and build a practical response you can repeat under load.

Where did this pattern show up this week?

List specific moments at work, at home, and in transition.

What did it cost you when it took over?

Name impact on body, mood, decisions, and relationships.

What two actions will you run for the next 7 days?

Choose actions small enough to use on hard days.

1. Trigger:

What activated the pattern?

2. Early body signal:

What did your body do first?

3. Reset action used:

What fast action lowered intensity?

4. Better next response:

What exact behavior will you use next time?

> Track this daily for one week, then tighten the plan around what actually worked.

Worksheet 2: Worksheet: Family Life in Transition: Moves, Shifts, and New Normals - Partner/Team Check-In

Use this page for one short practical check-in each week.

What do others notice first when this pattern is active?

Capture observable behavior, not labels.

What support helps most and least?

List what helps and what escalates the issue.

What agreement will you both practice this week?

One clear action from each side.

1. Signal phrase:

What phrase means pause and reset?

2. Time-to-return commitment:

When will re-engagement happen?

3. Repair step:

What specific repair happens after a rupture?

> Keep check-ins short, specific, and repeatable.

Closing

You've seen and carried more than most people will ever know.

Blow ups and shutdowns are not proof that you're broken or that your relationship is doomed.

They are signs that your nervous system has been running in survival mode for a long time.

The fact that you picked up this guide and read it matters.

You are choosing to be intentional about how you show up for the people who matter to you.

That choice is a lot more powerful than having flawless days.

About the Author

Michael Jenkins, MS, MSW, LCSW

Founder, Never Forgotten Services, PLLC

Michael is a U.S. Army combat veteran and licensed clinical social worker who specializes in working with military, veterans, and first responders—our frontline folks. After leaving the military, he spent years fighting his own demons and trying to figure out why he could hold it together on the job but feel completely wrecked at home. The turning point was realizing he wasn't the only one living that split life—and that nobody should have to feel that alone, confused, or broken by what the job asks of them.

Since then, his work has focused on trauma, the impact of high-stress jobs on home life, and helping families make sense of what the job brings home. He uses plain language, zero shame, and a "you're not crazy, this actually makes sense" approach to help frontline folks and their families understand what's happening in their nervous systems and learn how to move through the chaos together.